Nab-paclitaxel · Days 1, 8 and 15 of 28

National Cancer Control Programme (NCCP) 00736 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Nab-paclitaxel on Days 1, 8 and 15 of each 28-day cycle, with a planned week off on Day 22. Continue while beneficial and tolerated. This off-label schedule requires specific consent.

STEP 1

Before treatment

full blood count (FBC), kidney and liver tests at baseline and before each cycle; NCCP explicitly lists a Day 8 FBC. The unit confirms count testing for every administration decision, including Day 15.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Nab-paclitaxel is infused over 30 minutes. Day 1, 8 and 15 visits follow their own count decisions.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Routine allergy premedicines are usually not required for nab-paclitaxel. Prior reactions require individual review; treatment is still given where a serious reaction can be managed.

Sources: NCCP 00736 v2 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.